

The product

What we sell, what it costs, where it sits, what it earns.

The figures in this work have moved repeatedly, and the reason is that they were being derived from a product that had not been settled. This note fixes the product first. Pricing, positioning and financials then follow from it rather than the other way round.

01 / THE PRODUCT

Three revenue lines, two capture businesses.

Line	Sold to	What it is	Revenue
1. Panel and membership	Specialists	Access to rooms, infrastructure and an address. Panel for breadth, membership for the committed	Sessions and fees
2. Family practice	Families and corporates	Bundled annual care. The demand engine: it manufactures the flow the panel needs	Packages
3. Day case and aesthetics	Cash-pay patients	Aesthetics, dermatology, hair transplant. Defines the ambulatory identity	Procedure fees
Medbury Diagnostics	Everyone above	Laboratory and imaging	Per test
Medbury Pharmaceuticals	Everyone above	Dispensing and retail	Per script

Line 2 is the one that makes the others work. A panel of specialists with no patients is an empty building. The family practice generates first-contact volume, refers into the panel, and drives the diagnostics and pharmacy capture. It is the only line Lyfe Place controls end to end, and it should be built first.

02 / LINE 1

Panel for breadth, membership for the committed.

Tier	What it is	What it does
Panel	Credentialed, listed in the directory, books sessions at guest rates. No minimum, no lease, small annual administration fee	Breadth. Fills the specialty map cheaply and carries no risk
Membership	Everything above, plus member session rates, priority booking, named room preference, directory prominence and the lounge	Depth. For the two-plus sessions a week cohort, where the discount pays for the fee

The upgrade is self-selecting. At a 33% member discount and a NGN 2.5M fee, a consultant breaks even at roughly two sessions a week. Below that the panel is better value and above it membership is. Nobody has to be persuaded, and nobody is oversold.

Lyfe Place markets the campus and that is what fills diaries. Campus marketing, family practice referrals, diagnostics walk-in traffic and corporate accounts are all real and all funded. But they appear in the sale as what the campus does, **never as a patient volume a member is owed.** A volume promise creates a liability nobody can control and it will be produced against you at the first quiet quarter.

03 / LINE 2

Family practice, bundled and packaged.

Lyfe Place's own operating company, not a tenant. It launches in **one first-floor consulting room** and takes a second when member numbers justify it. The ground floor has no room for it: every space there is committed to arrival, diagnostics or the theatre suite. The first-floor phlebotomy draw point serves it, so the screening product keeps its diagnostics adjacency.

Package	What is included	NGN a year
Individual	Unlimited GP access, annual screen, vaccinations, chronic disease management, quarterbacked referral into the panel	600,000
Family, up to four	As above for the household, with paediatric cover	1,500,000
Corporate, per head	Executive screen, occupational health, priority access	400,000

Quarterbacked referral is the point. The family physician does not just refer, they book the specialist, order the tests in advance and hold the thread. That is what a package buys that a consultation does not, and it is what routes the patient into the panel, the laboratory and the pharmacy rather than out of the building.

04 / LINE 3

Aesthetics and day case, as the anchor.

Sub-line	What is done	Where
Aesthetic medicine	Injectables, fillers, peels, microneedling, laser hair removal and resurfacing, HIFU, cryolipolysis, PRP, skin boosters	Treatment room
Aesthetic surgery, local	Mole and lesion excision, earlobe repair, scar revision, upper-lid blepharoplasty, thread lifts, labiaplasty	Clean procedure room
Dermatology	Medical and cosmetic, with skin cancer excision and biopsy	Treatment room
Hair transplant	FUE, six to ten hour single-patient cases under local anaesthesia	FUE suite

The theatre, and what it costs to have one

A day-case theatre goes on the **ground floor**. I had ruled a theatre out entirely, and that was wrong: the objection was evacuation, and it only applies above ground level where a sedated patient cannot be moved horizontally. At ground level with trolley access to an ambulance, a theatre is an engineering and cost question rather than a safety stopper.

The suite	sqm	What it takes
Theatre	24.7	The largest ground floor room, level access to the exterior
Recovery, 2 monitored bays	19.3	Was the plaster-capable consulting room
Scrub and prep	10.4	Was the talking-consultation room
Sterile store and dirty utility	7.8	Was imaging reporting, which folds into ultrasound
The suite	62.2	

Capital, NGN M	Amount
Ventilation: AHU, HEPA, ductwork, external plant	25 to 40
Theatre lighting, pendant, operating table	25 to 40
Anaesthetic machine and full monitoring	15 to 25
Scrub, sterile store, dirty utility fit-out	12 to 18
Isolated power supply panel and UPS	8 to 15
Recovery, two monitored stations	8 to 12
On top of the NGN 232M fit-out	93 to 150

The trade, stated plainly. The theatre unlocks the surgical half of the anchor: liposuction, abdominoplasty, breast augmentation and reduction, rhinoplasty, facelift and gynaecomastia, plus general day surgery and gynaecology under general anaesthesia. None of it can be done any other way. **The cost is NGN 93M to 150M, every consulting room on the ground floor, and the plaster room with its X-ray adjacency.** Campus fit-out goes from NGN 232M to between NGN 325M and NGN 382M.

Hair transplant is the best single fit for this building. Local anaesthesia only, so no theatre and no anaesthetist. One patient occupies the suite all day, so it produces **one arrival** and relieves the single-reception constraint instead of loading it. And the ticket runs NGN 1.2M to 4M. Nothing else on the campus converts a room into revenue at that rate.

05 / WHAT THE PANEL COVERS

Specialties, and what enables each.

Enabled by	Specialties
Digital X-ray	Orthopaedics, adult: fracture clinic, osteoarthritis, sports injury, pre and post-operative review Paediatric orthopaedics: see below Rheumatology, respiratory with spirometry
Ultrasound	Obstetrics and gynaecology, fertility, general surgery for hernia and gallbladder, vascular including varicose veins and sclerotherapy, musculoskeletal and sports, paediatrics, urology
Echocardiography	Cardiology, and pre-operative cardiac clearance for the surgical aesthetic work, which the anchor needs anyway
Laboratory	Endocrinology, nephrology, haematology, infectious disease, gastroenterology, fertility, and the family practice's screening programme
Clean procedure room with an anaesthetist	Endoscopy, upper GI and flexible sigmoidoscopy. Hysteroscopy. Cystoscopy. Image-guided pain injections. Vasectomy
No diagnostics needed	Psychiatry, psychology, neurology, palliative care, dietetics, occupational health, genetics

Paediatric orthopaedics, which X-ray unlocks specifically

This deserves separating out because it is high-volume, clinic-based, needs no theatre, and is materially under-provided in Nigeria.

Condition	What is done here
Ponseti clubfoot correction	Serial casting over six to eight weeks, then bracing. Clinic-based, no theatre, life-changing, and there is very little of it available. The strongest single paediatric proposition on the campus
Developmental hip dysplasia	Ultrasound screening in infants, X-ray after six months, Pavlik harness fitting and review
Limb deformity	Genu varum and valgum, torsional profiles, leg length
Scoliosis	Screening, monitoring and brace review
Perthes and SCFE	Diagnosis and serial monitoring
Fracture follow-up	Cast changes, removal, review films

One fit-out addition unlocks all of it: a plaster trap and a cast saw. Serial casting blocks ordinary drainage, so the sink needs a plaster trap, and cast removal needs a saw and somewhere the noise does not carry into consulting. Site it in the **ground floor consulting room, next to X-ray**, so the check film, the cast and the review film all happen in one place. The cost is a few hundred thousand naira and it opens an entire specialty.

06 / PRICING

Three tariffs, one principle.

The principle: **price the access to fill it, and take the margin on what the flow generates.** Sessions and packages are priced to be said yes to. Diagnostics, pharmacy and procedures are where the money is made.

Sessions, NGN	Member	Panel or guest
Early, 07:00 to 09:00, 2h	44,000	66,000
Daytime, 09:00 to 13:00, 4h	61,000	91,500
Afternoon, 13:00 to 17:00, 4h	61,000	91,500
Evening, 17:00 to 21:00, 4h	94,000	141,000
Treatment room, per session	180,000	270,000
Clean procedure room, sedation-capable	480,000	720,000

Fees, NGN a year	Price	Session rate	What it buys
Panel	400,000	Guest rates	Credentialing, directory listing, booking access. Recovered in four sessions
Associate	1,200,000	20% off guest	Pays for itself at 0.8 sessions a week

Full	2,400,000	35% off guest	Priority booking, named room preference, lounge. Pays for itself at 1.2 a week
Fellow	4,200,000	35% off guest	All of the above, plus a standing weekly slot, directory prominence and inclusion in campus marketing
Group	2,000,000 each	35% off guest	Three or more from one practice. A shared named room, pooled blocks, one invoice. A 17% discount on Full for bundling
Partnership	Revenue share	n/a	Not a membership. For anchor service lines: Lyfe Place provides the room, equipment and marketing, the clinician provides the practice, and the revenue is shared. This is how the hair transplant suite and the theatre should be structured
Family practice, individual	600,000		
Family practice, family of four	1,500,000		
Family practice, corporate per head	400,000		

Two corrections to earlier pricing. The panel was NGN 150,000, which is recovered in 1.4 sessions and is not a filter but a rounding error. It has to pay for MDCN verification, indemnity checks, references, annual re-verification and the insurance exposure of having someone on the premises, so **NGN 400,000**. And a single membership tier was leaving money and members on the table, so there are now three.

The ladder now runs from occasional user to co-venturer. Panel for someone who wants the address occasionally. Associate, Full and Fellow for individuals in rising order of commitment. **Group** for a practice of three or more who want a shared room and one relationship, which is how orthopaedic and obstetric practices actually operate. And **Partnership**, which is not a membership at all: for the anchor lines, Lyfe Place puts up the room, the equipment and the marketing, the clinician puts up the practice, and the revenue is shared. **That is the right structure for the hair transplant suite and the theatre**, because neither works as a room somebody rents.

Tiering does not raise the price, it widens the funnel. Run as one tier, the fee that breaks even at a committed 1.5 sessions a week is about NGN 2.5M, which is roughly where we already were. The problem is not the number, it is that it asks every specialist to commit at the same level. The ladder lets someone join at NGN 1.2M who would never have joined at NGN 2.4M, and lets the busiest pay NGN 4.2M for a standing slot they would otherwise have to fight for each week. Every tier pays for itself at a usage level the member can check against their own diary, which is what makes it an easy sale rather than a persuasion.

Guest and panel rates are set at 1.5x member. The evening rate is the one most exposed: at NGN 23,500 an hour it sits 29% above our Lagos benchmark, and there is about NGN 21,000 of room to concede if the survey says so.

07 / POSITIONING

What it is, and what it is not.

It is	A private ambulatory campus where senior specialists hold sessions at a good address, families buy bundled care, and aesthetic and day-case work anchors the identity. Diagnostics, pharmacy, procedure rooms and a day-case theatre on site, so a patient is worked up, operated on and home the same day
It is not	A hospital. There is a day-case theatre, but no beds, no emergency department, no intensive care and no overnight stay . Every patient goes home the same day. It does not compete with the tertiary centres and should never be described as though it does
For specialists	The address and the infrastructure, without a lease. Turn up, practise, be paid, leave. Somebody else handles reception, records, billing, collection and the nurse
For patients	One building, one visit. Seen, scanned, tested, prescribed, without crossing town between each step
For Medbury	A medical park under the Medical Infrastructure Division, earning from access, from packages, and from every test and script the flow generates

08 / FINANCIALS

What it earns, on the empanelled model.

NGN M a year, stabilised	Low	Base	High
Members, all tiers	18	31	48
Panel specialists	30	45	60
Room fill	20%	38%	65%
Sessions and fees	181.1	342.2	583.7
Procedure and treatment rooms	101.6	101.6	101.6
Family practice packages	64.0	128.0	192.0
Campus revenue	346.7	571.8	877.3
Fixed cost base, including family practice	(230.5)	(230.5)	(230.5)
Contribution	116.2	341.3	646.8
	Low	Base	High
Total capital	527	527	527
Break-even, share of campus revenue	66%	40%	26%
Payback from stabilisation	4.5 yrs	1.5 yrs	0.8 yrs

Empanelment gives a lower number with far better odds of hitting it, and that is the right trade. The earlier single-tier model needed 60 specialists each paying NGN 2.5M, which is a lot of persuasion. This needs 25 members and 45 on a panel at NGN 400,000, which is a much easier sale and a much shorter path to breadth. **Everything turns on the member count, which is precisely what the survey measures.**

One capacity consequence to hold in view. With the theatre taking the ground floor and the family practice taking a first-floor room, the sessional pool falls to **six rooms**, all upstairs. At a practical booking ceiling of about 75%, that is roughly 4,970 blocks a year. It carries the base case comfortably and about half as much again, but **the high case does not fit in it.** The theatre buys surgical revenue and caps sessional revenue, and both effects are real.

The family practice line is indicative and needs its own model: panel size, physician ratios, utilisation and renewal rates all matter and none has been worked. It is shown here because leaving it out understates the campus, not because the number is settled.

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Session rates are derived from our Lagos work less 10% and are unvalidated for Abuja. Member and panel counts, family practice packages and fill assumptions are what the consultant survey exists to test. Capital and cost base are carried from the fit-out and forecast note. Not a binding offer, and not legal or tax advice.